Proposition One: Reproductive Freedom & Privacy Act ★ ★ ★

Short Ballot Title

Measure creating right to abortion before fetus viability, and post-viability to protect health; right to privacy; healthcare provider liability protections.

Long Ballot Title

The measure seeks to change Idaho's laws by introducing a right to reproductive freedom and privacy including a right to abortion up to the point of the fetus's ability to survive outside the womb. After fetal viability, there would be no general right to abortion except in cases of "medical emergency." The "medical emergency" exception would expand Idaho's current life exception and allow abortions when pregnant women face complicating physical conditions that threaten their life or health, "including serious impairment to a bodily function" or "serious dysfunction of any bodily organ or part."

The proposed measure codifies a right to make reproductive decisions, including contraception, fertility treatment, and prenatal and postpartum care. This includes a "right of privacy" in making these decisions. The measure seeks to prevent the state from enforcing certain abortion laws protecting the life of the unborn child. It would also impose a requirement that any restrictions on reproductive decisions, including abortion prior to fetus viability, must be "narrowly tailored to improve or maintain the health of the person seeking reproductive health care." The measure would also prevent the state from penalizing patients, healthcare providers, or anyone who assists in exercising the proposed right.

WHAT YOUR VOTE WILL DO ★ ★ ★

Yes

A **YES** vote would support creating a right to abortion before fetal viability—defined as a fetus' ability to survive without extraordinary medical measures—and after fetal viability in cases of medical emergency; providing protections against professional discipline and civil and criminal liability for healthcare providers; and codifying a statutory reproductive right to freedom and privacy.

No

A **NO** vote would support making no change to Idaho's current law, which preserves the life of preborn children by prohibiting abortion, except when necessary to prevent the death of the pregnant woman, and during the first trimester in documented cases of rape or incest reported to law enforcement.

FUNDING SOURCE STATEMENT ★ ★ ★

Provided by Idahoans United for Women & Families

No funding source is required for the Reproductive Freedom and Privacy Act as it does not create any new financial obligation on the state. Because no funding source is required, the Act has no impact on income taxes, sales tax, or product taxes.

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FISCAL IMPACT STATEMENT ★★★

Provided by the Idaho Division of Fiscal Management

Summary of Fiscal Impact

The State estimates the initiative would increase state expenditures between $3,100 and $7,800 annually, less than 0.001% of the state share of Idaho's Medicaid budget. This impact is derived from the costs of treating chemical abortion complications for women enrolled in Idaho's Medicaid program. Medicaid covers medically necessary services to treat complications from all abortions. It is anticipated that as legal abortions increase, the complications will also increase. This will likely result in an increase in Medicaid covered services and expenditures to treat complications from chemical abortions, which the State has reliable and readily available data to support.

Detailed Statement of Fiscal Impact Including Assumptions

Based on the amount of reliable data gathered by the Division [sic] Financial Management (DFM) and the outcome of the initiative (if passed), DFM limited its assumptions about any increase in costs to the state to those costs arising from the treatment of Medicaid recipients for chemical abortion complications. DFM does not have reliable and readily accessible data to reasonably calculate the fiscal impact attributable to the complications for surgical abortions.

Based on data provided by Department of Health and Welfare (DHW), DFM considered the number of childbearing women on Medicaid in 2021 which was 97,055. This represents about 25% of the total Idaho population for women in this same age group. DFM assumed that, as a result of this initiative, the number of chemical abortions conducted annually would increase to the levels prior to 2022, the year of the U.S. Supreme Court's decision returning the regulation of abortion to the States. Based on the number of chemical abortions performed in Idaho using the 2021 data from the vital records report (approximately 1,180), DFM assumed 25% (approximately 300) of those individuals were Medicaid recipients.

Taking the number of Medicaid recipients who would receive a chemical abortion, DFM assumed that 2% - 5% of those chemical abortions would result in complications. DFM assumed this based on information provided in (1) the United State's [sic] Food and Drug Administration's warning label for the Mifeprex and (2) studies related to abortion complications that were published on the National Library of Medicine website, which DHW provided to DFM.

Based on the claims for complications from chemical abortions provided by DHW for 2019 and 2022 claims data, DFM applied an average per person claim cost and added a 4% medical inflation year over year to account for current medical costs in FY2026. Based on that calculation, DFM assumes that about 300 individuals on Medicaid in Idaho could receive a chemical abortion and of those, 6 to 15 individuals could have complications from that chemical abortion with an average claim cost of about $531 (state portion of the cost). Based on the process utilized, evidence readily available and gathered, and assumptions used to create the fiscal impact statement, total expenditures for the state would range from $3,100 to $7,800 per year.

While the specific fiscal impact to the state's expenditures is difficult to quantify, DFM cannot in good faith conclude that the proposed initiative will have no fiscal impact on state expenditures.

PRO - ARGUMENTS FOR PROPOSITION ONE ★ ★ ★

Provided by Idahoans United for Women & Families

This Proposition creates needed exceptions to Idaho's extreme abortion ban, ensuring that sensitive, urgent decisions in medical emergencies are made by patients and their doctors — not by the government.

When a pregnancy becomes dangerous, when a diagnosis is fatal, or when a mother's health is at risk, decisions about the best course of care and action should be made at her bedside, by her, her doctor, and her family. Her care should not be dictated by politicians who will never know her name.

Here is what too many Idahoans don't yet realize: Idaho's abortion ban is one of the most severe in the nation, and it took effect only recently. The impacts of that ban have been swift and dangerous.

The current law provides no health or emergency exception, leaving doctors unable to act until a crisis becomes critical. Doctors face prison time for using their best medical judgment. Pregnant women have been airlifted to Utah to receive health-saving interventions, or forced to wait until they are near death before receiving care.

- Voting YES would restore the standard Idaho operated under for nearly 50 years, with health and life exceptions to the abortion ban, including cases of fatal fetal diagnosis.
- Voting YES would protect doctors and nurses from criminal prosecution for providing standard medical care, at a time when Idaho is already losing OB-GYNs, family doctors, and emergency physicians.
- Voting YES also protects the care Idaho families count on every day: birth control, contraception, and fertility treatments like IVF.
- Voting YES does NOT create public funding for abortion, nor does it force any provider to violate their conscience.
- Voting YES does NOT change Idaho's current parental consent laws: all healthcare for minors would still require the permission of a parent or guardian.

As the titles and language of the Act plainly state, voting YES decriminalizes abortion, while maintaining reasonable restrictions, to ensure that the government isn't involved in personal family medical decisions. This is not a new idea: it is the same commonsense standard Idaho doctors practiced under for two generations.

The signers of this Act — thousands of people from all walks of life in every corner of our state who put this question on the ballot for your vote — agree on one thing even when they may agree on little else: politicians are the wrong people to be making sensitive, private, complex decisions about our families, health, and futures.

There is one question every voter should ask themselves: who do you trust with your family's most private medical decisions? Is it you and your doctor, or politicians?

The only way to ensure there are exceptions to Idaho's ban is by voting YES on Proposition One.

CON - ARGUMENTS AGAINST PROPOSITION ONE ★ ★ ★

Provided by the Idaho Family Policy Center, Idaho Chooses Life, Right to Life Idaho, Too Extreme for Idaho, and the Roman Catholic Diocese of Boise

Proposition 1 is a backdoor attempt to legalize late-term abortions, long past the point in pregnancy at which babies can feel pain. Most people don't support elective abortion at seven or even eight months of pregnancy. Most people want parents involved when their teenage daughter is seeking an abortion. And most people believe that abortions should only be performed by doctors. Prop 1 erases all of these protections and goes too far for most Idahoans.

When it comes to writing laws, words matter—and the lawyers who wrote Prop 1 deliberately used tricky language to mislead voters. It's deceptive by design. And voters must examine the fine print.

Prop 1 claims to allow abortions only to the point of "viability" but throws out the longstanding definition of that term. Idaho law has always defined "fetal viability" as the point at which a baby could "potentially live outside the mother's womb, albeit with artificial aid." But Prop 1 redefines viability as the point when a baby has a "significant likelihood of sustained survival without extraordinary medical measures." Notice the bait-and-switch: the word "potentially" has been replaced by "significant likelihood," and the phrase "albeit with artificial aid" has been replaced by "without extraordinary medical measures."

This radical approach isn't based in law or medicine. Many premature babies can survive with the assistance of oxygen, nutrition, or other extraordinary measures. But under Prop 1, some of these babies would no longer be protected. Prop 1 would permit abortion on demand until seven or eight months of pregnancy—far beyond what was allowed even under Roe v. Wade.

Almost everyone agrees that there comes a time in a pregnancy when it's just too late for an abortion. Good people may disagree over when to draw that line. But most Idahoans believe unrestricted late-term abortion is too extreme for Idaho.

And it doesn't stop there. Prop 1 also eliminates the longstanding safety requirement that only doctors can perform abortions. It replaces "physician" with the vague term "health care provider," which it defines as any "licensed person or entity that provides health care or medical treatment." This even includes people who aren't doctors—such as abortion clinic employees. By stripping women's health protections, Prop 1 endangers the women it is supposed to help.

But perhaps the most shocking of all? Prop 1 authorizes "health care providers" to perform abortions on teenage girls behind the backs of their parents, overturning parental consent requirements. That's because Prop 1 creates a right to abortion for "every person" regardless of age, making abortion the only procedure that could be performed on a child without parental involvement.

Prop 1 organizers want to bring California-style abortion laws to Idaho. They are determined to legalize late-term abortions. And they won't stop until they've gutted patient protections and eliminated parental rights.

That's what they won't tell you—but it's the truth. And it's right there in black and white.

Prop 1 is too extreme for Idaho. Vote NO.

REBUTTAL ARGUMENTS ★ ★ ★

Rebuttal to Pro Arguments

Provided by the Idaho Family Policy Center, Idaho Chooses Life, Right to Life Idaho, Too Extreme for Idaho, and the Roman Catholic Diocese of Boise

Abortion advocates want you to believe that Prop 1 simply takes us back to Roe v. Wade. But in reality, Prop 1 is far more extreme—authorizing late-term abortions well beyond what was allowed under Roe v. Wade. And that's just the beginning.

Prop 1 organizers claim that it doesn't eliminate current parental consent laws. But the proposition language says otherwise. Prop 1 creates a "right" to abortion for "every person" regardless of age—making abortion the only procedure that children can get without parental involvement.

Prop 1 also contains a provision that would override other state laws, including parental consent requirements and another state law that protects your tax dollars from funding abortion. Their lawyers didn't have to write it that way. But they did—and they're lying to you about it.

And their claim that Prop 1 is necessary to protect birth control and IVF? Also untrue. These services are already legal in Idaho—and Prop 1 isn't needed to protect access.

Are Idaho laws dangerous for mothers? No. Idaho laws allow abortions to save the life of the mother and in cases of rape and incest—and provide ample latitude for physicians to perform medically necessary care in accordance with their good faith medical judgment.

As for the doctor shortage? More deception. Actively licensed OB/GYNs in Idaho have actually increased by over 20%, according to the Idaho Board of Medicine.

Voters deserve to know the truth: Prop 1 is just too extreme for Idaho. Vote NO.

Rebuttal to Con Arguments

Provided by Idahoans United for Women & Families

For a true picture of what Proposition One does (and doesn't do), you can read the official ballot summary — written by Idaho's conservative Attorney General and reviewed by Idaho's Supreme Court for accuracy. The language plainly shows that Proposition One is a reasonable way to stop Idaho's dangerous abortion ban from putting women and families at needless risk.

Proposition One Restores Exceptions to Idaho's Extreme Ban.
Countless things can go wrong in pregnancy, and doctors must be able to offer critical, health-and-lifesaving care to women without the risk of prison. That was Idaho's standard of care for decades, and Proposition One restores it.

Proposition One Includes Common-Sense Restrictions on Abortions.
Abortion remains illegal after viability under Proposition One, except when a doctor determines it is medically necessary to protect the woman's life or health. That's the restriction that we followed for 50 years, the one most Idahoans want, and it's the same doctor-determined standard used in states like Utah.

Proposition One Leaves Parental Rights Protected and Unchanged.
Parents' rights are and will remain among the most strongly protected constitutional rights, and no doctor in Idaho delivers healthcare to minors without the express permission of their parent or guardian.

Proposition One Keeps the Same Definition for Physicians.
Prop One's definition of "physician" comes directly from existing Idaho Code and, like current law, recognizes that nurse practitioners and physician assistants legally provide reproductive care all the time. That's standard medicine.

Read it for yourself: Proposition One restores common-sense healthcare to Idaho.

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FULL TEXT OF PROPOSED LAW FOR PROPOSITION ONE ★★★

Be it enacted by the People of the State of Idaho:

SECTION 1. That Title 39, Idaho Code, be, and the same is hereby amended by the addition thereto of a NEW CHAPTER, to be known and designated as Chapter 8, Title 39, Idaho Code, and to read as follows:

39-801. SHORT TITLE. This act shall be known and may be cited as the "Reproductive Freedom and Privacy Act."

39-802. STATEMENT OF PURPOSE. The Reproductive Freedom and Privacy Act recognizes that reproductive health care choices—such as the use of contraception, fertility treatments, childbirth care, miscarriage care, the decision to continue one's own pregnancy, and abortion—are deeply private matters that should be decided by a person in consultation with their health care provider. This statute upholds a person's rights to make their own decisions based on their own values, health care needs, and circumstances—free from the fear of external pressures or punitive consequences to them or their health care provider. The act supports a person's right to reproductive freedom and privacy, protects the confidential nature of the patient-provider relationship, and secures a person's right to make their own health care decisions without government interference.

39-803. REPRODUCTIVE FREEDOM AND PRIVACY ACT

1. This act establishes a right to make private reproductive health care decisions, including abortion up to fetal viability and in medical emergencies.

2. Notwithstanding any other provision of law to the contrary:

   a. Every person has the right to reproductive freedom and privacy, which is the right to make personal decisions about reproductive health care that directly impact the person's own body, including but not limited to the right to make decisions about:

      i. Abortion;
      ii. Childbirth care;
      iii. Contraception;
      iv. Fertility treatment;
      v. Miscarriage care; and
      vi. Prenatal, pregnancy, and postpartum care.

   b. The right to reproductive freedom and privacy includes the right of privacy in making personal decisions about reproductive health care in consultation with a health care provider.

   c. A person's voluntary exercise of the right to reproductive freedom and privacy shall not be burdened, interfered with, discriminated against, deprived, or prohibited by the state, directly or indirectly, in any manner, unless such state action is narrowly tailored to improve or maintain the health of the person seeking reproductive health care through the least restrictive means.

   d. Any person or entity may voluntarily advise, assist, facilitate, inform, refer, or otherwise aid another person exercising the right to reproductive freedom and privacy, and the state shall not burden, interfere with, discriminate against, deprive, or prohibit such acts, directly or indirectly, in any manner, unless such state action is narrowly tailored to improve or maintain the health of the person seeking reproductive health care through the least restrictive means.



e. In no case may reproductive health care provided consistent with this act by a health care provider be a basis for professional discipline, civil liability, or criminal liability as to a health care provider solely on the basis that the health care provider knowingly advised, assisted, facilitated, informed, referred, or otherwise aided a person in exercising their right to reproductive freedom and privacy.

3. Provided further that as this act specifically applies to abortion:

   a. After the point of fetal viability, it shall not be a violation of the right to reproductive freedom and privacy for the state to regulate abortion, except in cases of medical emergency.

4. The provisions of this act are to be liberally construed in favor of reproductive freedom and privacy and are intended to control over any other section of Idaho Code, consistent with the following:

   a. Nothing in this act shall be construed to limit any right or access to reproductive health care, including but not limited to abortion, that currently exists or is otherwise provided for or guaranteed by law.

   b. This act does not create a financial obligation on the state, its agencies, or their programs to pay for, fund, or subsidize the reproductive health care protected by this act.

   c. Nothing in this act will be deemed to bar or otherwise apply to a claim of medical malpractice against a health care provider for failing to comply with the applicable community standard of health care practice, as set forth in Section 6-1012, Idaho Code.

   d. Nothing in this act will infringe on the protections and accommodations regarding a health care provider's freedom of conscience, as set forth in Section 18-611, Idaho Code.

   e. If the application of any provision of this act is declared invalid for any reason including by the application thereof, such invalidity shall not affect the validity of the remaining portions of the act that can be given effect without the invalid provision or application, and to this end the provisions of this act are severable.

5. Definitions. As used in this act:

   a. "Abortion" means a medical treatment that is intended to terminate a pregnancy.

   b. "Childbirth care" means the medical treatment provided by health care providers in the processes of labor and delivery, including all stages of labor, the act of giving birth, and any medical procedures related to the delivery of a child, whether by vaginal birth or cesarean section.

   c. "Contraception" means any act of preventing pregnancy including the use of any device, drug, procedure, or biological product intended for use in the prevention of pregnancy.

   d. "Fetal viability" means the point in a pregnancy when, on the basis of a physician's good faith medical judgment, based on the facts known at the time, and determined on a case-by-case basis, the fetus has a significant likelihood of sustained survival outside of the uterus without extraordinary medical measures.



e. "Fertility Treatment" means the treatment of infertility and related conditions, including but not limited to assisted reproductive technology and in vitro fertilization.

f. "Health care provider" means a licensed person or an entity that provides health care or medical treatment.

g. "Medical emergency" means a physical medical condition that, on the basis of a physician's good faith medical judgment, based on the facts known at the time, and determined on a case-by-case basis, complicates the physical medical condition of a pregnant patient as to warrant an abortion:

   i. To protect a pregnant patient's life; or

   ii. For which a delay may:

      a. Place the health of a pregnant patient in serious jeopardy;

      b. Cause serious impairment to a bodily function of a pregnant patient; or

      c. Cause serious dysfunction of any bodily organ or part of a pregnant patient's body.

h. "Miscarriage care" means the treatment and management of pregnancy loss.

i. "Physician" means a person licensed to practice medicine and/or surgery or osteopathic medicine and surgery in this state as provided in Chapter 18, Title 54. A physician is a health care provider as defined in this act.

j. "Prenatal, pregnancy, and postpartum care" means health care and other medical services provided before, during, and after childbirth, including but not limited to exams, treatments, diagnostic testing, postpartum recovery and support, and any other care necessary for the health of the patient.

k. "Reproductive health care" means health care and other medical services related to the reproductive processes, functions, and systems. It includes but is not limited to abortion, contraception, childbirth, fertility treatment, miscarriage care, and prenatal, pregnancy, and postpartum care.

SECTION 2. This act shall be in full force and effect on and after January 1, 2027.

ABSENTEE VOTING ★ ★ ★



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"The Americans are the first people whom Heaven has favoured with an opportunity of deliberating on, and choosing the forms of government under which they should live."

-John Jay, first Chief Justice of the United States